



Clinical Practice Guideline

In-hospital resuscitation

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All Clinical Staff

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Introduction

The goal of in-hospital resuscitation is to attempt to prevent cardio-respiratory arrest by early identification and intervention using the ABCDE approach to identify and assess clinical deterioration¹.

These guidelines are aimed primarily at medical and nursing healthcare professionals who are first to respond to an in-hospital cardiac arrest or other medical emergency.

Risk Statement

Non-compliance with this guideline may:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☐	Impact on Patient Safety	☒
Breach professional standards	☐	Misconduct	☐
Breach SCGH Mission & Values	☐	Staff Safety	☒
Other:	☐		

Procedure

After in-hospital cardiac arrest the division between basic life support (BLS) and advanced life support (ALS) is arbitrary; in practice, the resuscitation process is a continuum, for all in-hospital medical emergencies, including cardiac arrest, follow the SCGOPHCG in-hospital resuscitation algorithm, ensure that:

- cardiorespiratory arrest is recognised immediately,
- help is summoned locally, and a met call activated by dialling 55,
- cardiopulmonary resuscitation (CPR) is started immediately using adjuncts, for example an oropharyngeal airway (OPA) and bag valve mask (BVM), and, if indicated, defibrillation attempted as rapidly as possible and certainly within 3 minutes. Details should be recorded after each event.

1. Ensure Personal Safety

Personal safety is the first priority during any resuscitation attempt.

- Check that the surroundings are safe.
- Don gloves as soon as possible.
- Personal protective equipment (PPE) (eye protection, face masks, aprons, and gowns) may be necessary as per standard and transmission-based precautions and should be donned prior to touching the patient and prior to entering an isolation room.
- Be careful with sharps; a sharps box must be available as soon as possible.
- Use safe handling techniques with appropriate equipment and a planned team approach. It may be appropriate to commence CPR on the floor and await equipment.

2. Check the Patient for a Response

When a healthcare professional sees a patient collapse or finds a patient apparently unconscious or deteriorating in hospital, they should assess the environment for dangers then assess their level of consciousness by trying to elicit a verbal response.

If the patient does not respond firmly squeeze their shoulders and ask a question that

should elicit a response such as 'Are you all right or can you open your eyes?' If they do not respond or you are concerned, send for help.

It will be possible to undertake several actions simultaneously if other members of staff are nearby.

2.1 Utilise the SCGH In-hospital Resuscitation Algorithm (see appendix 1)

2.1.1 If the patient responds

Urgent medical assessment may still be needed. Send for help according to the SCGH or OPH Observation and Response Chart (ORC) / MET escalation protocol. This may be the SCGH Medical Emergency Team (MET) or admitting team depending on escalation criteria / location of the individual.

While waiting for the team assess the patient using the ABCDE (airway, breathing, circulation, disability, and exposure) systematic approach and manage the patient within the responder's scope of practice (see [appendix 5](#)).

- Assess airway –
 - Is the patient talking?
 - If so, the airway is likely to be clear.
 - Are there abnormal sounds like stridor or gurgling?
 - The airway is likely to be partially obstructed.
 - Consider suctioning, airway opening manoeuvres or use of adjuncts and oxygen therapy.
- Assess breathing - respiratory rate, chest movement, effort of breathing and oxygen saturation levels - consider auscultation and ordering a chest x-ray, oxygen therapy and supplemental breaths using a bag and mask if breathing is slow (<5 bpm).
- Assess circulation - pulse rate, blood pressure, colour and capillary refill time, blood/fluid loss - consider intravenous access, fluids, and medications as prescribed, commencing cardiac monitoring and 12 lead ECG.
- Assess disability - assess response using AVPU (alert or responds to voice or pain or is unresponsive), check history/documentation, medication chart and blood glucose. Consider performing a Glasgow Coma Score (GCS) with pupillary response.
- Assess exposure - check temperature, urine output and pain score - consider head to toe examination for any injuries, rashes, bleeding or swelling, checking peripheral pulses/wounds/drains etc.

NB: Remember to always assess the effect of any intervention e.g., oxygen therapy or fluid resuscitation and treat life threatening problems as found.

Once initial assessment is completed continue to monitor / repeat ABCDE until help arrives. If the patient's condition changes repeat the assessment and review escalation as appropriate.

- Prepare to provide clinical handover to the team using the ISOBAR (identify, situation, observations, background, assessment, and recommendation) communication framework².

2.1.2 If the patient does not respond

- Send for help by requesting bystanders call 55 - Code blue (if this has not already been done). Some areas of the organisation require a 000 call and communications will provide advice on this once 55 is called.
- Turn the patient onto their back.
- Open the mouth to check for obstruction and clear if necessary.
- Open the airway using head tilt and chin lift.
- If you suspect that there is a cervical spine injury, open the airway using a jaw thrust. Maintaining an airway and adequate ventilation is the overriding priority in managing a patient with a suspected spinal injury. If this is unsuccessful, use just enough head tilt to clear the airway. Use manual inline stabilisation to minimise head movement if sufficient rescuers are available. Efforts to protect the cervical spine must not jeopardise oxygenation and ventilation.
- Keeping the airway open, look, listen, and feel to determine if the patient is breathing normally. This should be a rapid check and take less than 10 seconds: Look for chest movement, listen at the patient's mouth for breath sounds and feel for air on your cheek (agonal breathing is common immediately after cardiac arrest and is not normal breathing - it is a sign of cardiac arrest and should not be mistaken for a sign of life).
- Remove headboard (if in place) and provide adequate room at the head of the bed. If the patient is on an air-filled mattress, deflate as soon as possible.

NB: those experienced in clinical assessment may wish to assess the carotid pulse for less than 10 s. This should be performed simultaneously while checking for breathing. If unsure as to the presence of a pulse assume there is no pulse and commence CPR.

2.1.3 If the patient is breathing normally

Urgent medical assessment is still required; ensure the MET / local medical team is called by completing the SCGOPHCG ORC and escalating appropriately. While awaiting this team, assess the patient using the ABCDE approach.

The patient is at high risk of further deterioration and cardiac arrest and needs continued observation until the team arrives.

2.1.4 If there is no normal breathing

- One person starts chest compressions as others call the MET and collect the resuscitation equipment and a defibrillator.
- Give 30 chest compressions followed by 2 ventilations using a bag and mask (do not perform mouth to mouth during an in-hospital resuscitation attempt).
- If the bag and mask or personnel are not ready to give the 2 breaths after the initial 30 compressions then the first responder should continue with chest compression only CPR until this equipment and personnel have arrived, when a 30:2 CPR ratio can be achieved.
- Minimise interruptions and ensure high-quality compressions.
- The correct hand position for chest compression is the centre of the chest or the lower half of the sternum (avoiding the xiphoid-sternal process).
- The recommended depth of compression for adults is 5 - 6 cm and the rate is at least 100 compressions min⁻¹ (but not more than 120 min⁻¹)

- Allow the chest to completely recoil in between each compression.
- If available, use a prompt and/or feedback device to help ensure high quality chest compressions.
- The person providing chest compressions should change every 2 min, or earlier if unable to continue high quality chest compressions. This change should be done with minimal interruption to compressions, such as when the breath is being delivered.
- Tracheal intubation should be attempted only by those who are trained, competent and experienced in this skill. Waveform capnography should be available routinely for assisting to confirm tracheal tube placement and subsequent monitoring of an intubated patient.
- Once the patient's trachea has been intubated or a supraglottic airway device has been inserted, continue chest compressions uninterrupted (except for defibrillation or pulse checks when indicated), at a rate of 100-120 min⁻¹, and ventilate the lungs at approximately 6 - 10 breaths min⁻¹.
- Avoid hyperventilation (both excessive rate and tidal volume), which may worsen outcome.

2.2 Defibrillation

Only clinical staff that have attained competency in either manual or shock advisory defibrillation within 12 months should perform this procedure.

- Manual defibrillation - completed an Advanced Life Support course and attained annual competency.
- Shock advisory defibrillation - Attained Hospital Basic Life Support annual competency.

(For paediatric resuscitation, continue CPR until an ALS (manual mode) competent operator arrives to assess the rhythm and deliver the appropriate energy (4 J/Kg) if a shockable rhythm is identified. The use of paediatric pads are preferable.

- If no paediatric defibrillation pads are **immediately** available use as for an adult, ensuring the pads do not touch. Standard Adult pads are suitable for patients 8 years and older.)

When the defibrillator arrives, apply self-adhesive defibrillation pads to the patient and complete a rhythm check.

- The use of adhesive pads will enable more rapid assessment of heart rhythm than attaching ECG electrodes.
- If using a semi-automated external defibrillator (SAED) switch on the machine and stop CPR immediately before pressing the analyse button then follow the prompts.
- If shock is advised, visually ensure everyone is clear of the patient and the intention to shock is clearly announced before depressing the shock button whilst overlooking the patient area. Minimise delay in delivery of shock and recommence CPR immediately.
- If no shock advised follow prompts and recommence CPR if no signs of life are obvious.
- For manual defibrillation, minimise the interruption to CPR by continuing chest compressions whilst charging the defibrillator ensuring everyone else is clear and then once charged ensure compressions are ceased before evaluating the rhythm.

- If the rhythm is shockable (VF/VT) visually ensure everyone is clear of the patient and the intention to shock is clearly announced before depressing the shock button whilst overlooking the patient area. Minimise delay in delivery of shock and recommence CPR immediately and continue for a further 2 minutes unless signs of life present.
- If the rhythm is non-shockable (asystole / all other non-shockable rhythms) then ensure the charge is disarmed before either recommencing CPR for asystole or checking for signs of life (< 5sec) then recommencing CPR for all other non-shockable rhythms.

NB: if a team leader has been assigned then it is their responsibility to verify the patient's cardiac rhythm before deliverance of the shock or the disarming of the defibrillator.

- Continue rhythm checks every 2 minutes as above.
- Continue resuscitation attempt until the resuscitation team arrives or the patient shows signs of life. Once the MET arrive they should follow the universal algorithm for ALS (see [appendix 3](#)).
- Once resuscitation is underway, and if there are sufficient staff present, prepare intravenous cannula and drugs likely to be used by the resuscitation team (e.g. adrenaline 1 mg / amiodarone 300 mg).
- Identify one person to brief the resuscitation team leader; and
- Use a structured communication tool for handover (e.g. ISOBAR²). Locate the patient's records (if applicable) and ensure that they are available.

2.3 Emergency Medication

- Medication boxes at SCGH and OPH that are stocked and ready for use are sealed with a red tamper evident tag. These boxes are located in the resus trolleys. If required in an emergency the red tag is broken by hand.
- The ONLY time the box should be unsealed is during an emergency / MET.
- Once resuscitation is underway and there are sufficient staff present, ensure the following equipment and drugs are prepared;
 - Adrenaline 1 mg / 1 ml and 20ml saline flushes x 3.
 - IV giving set primed with normal saline.
 - IV cannulation equipment.
 - Use syringe labels or attached drug ampoule to syringe.
- If amiodarone is required in a cardiac arrest the dose is 300 mg in up to 20mL of 5% dextrose given as a bolus.
- For all other medications ensure clarification from the team leader about drug, dose and presentation.
- SCGH also has paediatric resuscitation trolleys containing medication boxes with different content to the adult drug box. These boxes are identified with a yellow label.
- The replacement process is the same for both paediatric and adult drug boxes. Hospital Service Assistants can transport these boxes only if sealed with the supplied yellow tamper evident seal.
- Once the medication box is finished with, seal the box with the included yellow

tamper evident tag. To replace, there are 2 processes:

- SCGH only - between 0800 and 1400 Monday to Friday. Obtain a new box from Inpatient Pharmacy, then return used sealed drug box to Inpatient Pharmacy.

OR

- Outside of these hours nursing staff to contact Anaesthetic Technician on 71907, to obtain a new box from Theatre and then return used sealed box to theatres

NOTE – relevant to SCGH only:

There is a medicine kit transfer log attached to each medication box. Staff are required to complete the log with their name/signature/designation/date/time and box details each time the box is collected, transferred and received.

Anaesthetic Dept and Pharmacy are unable to take receipt of an untagged drug box. If yellow tag not available – In hours Pharmacy will provide a replacement yellow tag. Out of hours, to be logged in local S4 book, locked in S4 cupboard and exchanged in hours.

- OPH - exchange medication boxes through Pharmacy

3. Background notes

3.1 Hospital and staff factors

The exact sequence of actions after in-hospital medical emergencies and cardiac arrest depends on several factors including:

- Location (clinical or non-clinical area; monitored or unmonitored patients).
- Skills of staff who respond.
- Number of responders.
- Equipment available.
- Hospital system for response to cardiac arrest and medical emergencies (MET).

3.2 Location

Monitored arrests are usually diagnosed rapidly. Ward patients may have had a period of deterioration and an unwitnessed arrest. Ideally, all patients who are at high risk of cardiac arrest should be cared for in a monitored area where facilities for immediate resuscitation are available. Patients, visitors, or staff may also have a cardiac arrest in non-clinical areas.

3.3 Skills of staff who respond

All clinical staff should be able to recognise cardiac arrest, call for help, and start resuscitation. Staff should only perform tasks that they have been trained to do. For example, staff in critical care and emergency medicine may have more advanced resuscitation skills than staff who are not involved regularly in resuscitation in their normal clinical role. Hospital staff who attend a cardiac arrest may have different competencies in managing the airway, breathing and circulation.

The ARC Advanced Life Support Level 1 (ALS1) course is aimed at the majority of healthcare professionals who attend cardiac arrests rarely but have the potential to be first responders or resuscitation team members. A study in the UK found that the number of cardiac arrest calls decreased while pre-arrest calls increased after implementing a programme that included ALS1 teaching in two hospitals⁴. This was associated with an

increase in initial survival after cardiac arrest and survival to discharge.

The ARC Advanced Life Support Level 2 (ALS Level 2) course is aimed at doctors and senior nurses working in critical care areas of the hospital and those who may be resuscitation team leaders. During training and clinical practice there should be a greater emphasis on non-technical skills (NTS) ⁵. These consist of situational awareness, decision making, teamwork / leadership and task management. Tools such as ISOBAR should be used to ensure rapid effective communication and handovers.

3.4 Annual refresher training and competency assessments

All SCGOPHCG clinical staff are required to be trained and proficient in either basic or advanced life support and following initial training an annual competency assessment is required. Training and assessment in BLS or ALS from other institutions should undergo a comprehensive Recognition of Prior Learning (RPL) process.

If after a competency assessment staff are deemed not yet competent, follow up training and reassessment must be facilitated within 10 working shifts for full time staff, or within 1 month for part time staff. In addition, if staff are required to have an ALS competency to work in a specific clinical area, they should refrain from escorting patients from these areas until retraining and competency is achieved. There may also be other area specific restrictions put on staff such as restricting on-call work and working in resuscitation/trauma bays of the emergency department.

3.5 Number of responders

The single responder must ensure that help is on its way. If other staff are nearby, several actions can be undertaken simultaneously. Hospital staffing tends to be at its lowest during the night and at weekends. This may influence patient monitoring, treatment and outcomes.

3.6 Equipment available

Equipment used for CPR (including defibrillators) and the layout of equipment and drugs should be standardised throughout the hospital. All resuscitation equipment / trolleys must be checked on a daily basis for ward areas and a minimum of weekly in other areas to ensure equipment and medications are ready for use.

Hospitals and teams that regularly treat cardiac arrests should have monitoring and equipment for transferring patients after they have been resuscitated. This includes portable monitors with a minimum of pulse oximetry, ECG, non-invasive blood pressure and waveform capnography for ventilated patients.

For ward staff at **SCGH** it is the responsibility of the shift co-ordinator to ensure,

- the used resuscitation trolley has been cleared of all sharps and body substance before being restocked
- the resuscitation trolley is restocked as per stock list either locally, or if equipment is not available, nursing staff complete 'shopping list' by inserting amounts required for each item on 'Resus Trolley Consumable Restocking Checklist'. HSA will retrieve required items from stocking cage (Level 2 G Block outside Post Anaesthetic Care Unit). Nursing staff to restock and seal the trolley.
- a sharps container is available on the resuscitation trolley,
- daily checks of the resuscitation trolley and defibrillator and that this is recorded and signed for.

For ward staff at **OPH** it is the responsibility of the shift co-ordinator to ensure:

- the used resuscitation trolley has been cleared of all sharps and body substance before being restocked using local processes,
- the resuscitation trolley is restocked according to the stock list provided,
- a sharps container is available on the resuscitation trolley,
- daily checks of the resuscitation trolley and defibrillator and that this is recorded and signed for.

NB: MET / code blue response at OPH may require transfer back to SCGH via SJA priority transfer.

3.7 Medical Emergency Team (MET) / Code Blue Response

- The MET is called should a patient's condition or observations trigger calling criteria. This may be via the MET criteria or via the ORC (See [appendix 4](#)). Members of the MET should have additional training in advanced life support and crisis resource management.
- In-hospital cardiac arrests are rarely sudden or unexpected. A strategy of recognising patients at risk of cardiac arrest may enable some of these arrests to be prevented or may prevent resuscitation attempts in those patients who are unlikely to benefit from CPR. Complete the Goals of Patient Care form for patients who have had ceilings of care discussions. The e-form is preferable and must be printed and placed in the patients' healthcare record.
- Medical emergencies can be stressful and potentially traumatic. Team members should try to debrief after each event based on the actions and data collected during the event.
- It is the responsibility of a nurse in attendance to record events on the SCGOPHCG MET Record (MR702), including a printout of the code summary where applicable and attaching it to the record.
- It is the responsibility of the MET medical and nurse leader to check and sign that the MET Record is correct.
- It is the responsibility of the MET nurse leader to phone ext. 74788 leaving the location of the MET, MET recipient's name, unit number (if known), ward/area, date and time after the event and to fax the MET record to 6457 3947).

3.8 Diagnosis of cardiac arrest

Many trained healthcare staff cannot assess the pulse sufficiently reliably to confirm cardiac arrest⁹. Agonal breathing (occasional gasps, slow, laboured, or noisy breathing) is common in the early stages of cardiac arrest and is a sign of cardiac arrest and should not be confused as a sign of life/circulation¹⁰. Delivering chest compressions to a patient with a beating heart is unlikely to cause harm¹¹. However, delays in diagnosis of cardiac arrest and starting CPR will adversely affect survival and must be avoided.

3.9 High-quality CPR

The quality of chest compressions during in-hospital CPR is frequently sub-optimal. The importance of uninterrupted chest compressions cannot be over-emphasised. Even short interruptions to chest compressions may affect outcome and every effort must be made to ensure that continuous, effective chest compressions are maintained throughout the resuscitation attempt.

The person providing chest compressions should be changed every 2 min or sooner if signs of fatigue are present, but without delay in chest compressions.

3.10 Defibrillation strategy

The length of the pre-shock pause (the interval between stopping chest compressions and delivering a shock) is inversely related to the chance of successful defibrillation.

For every 5-second increase in the duration of the pre-shock pause the chance of successful defibrillation reduces by about half, therefore it is important to minimise this pause^{12, 13} ensuring safety is maintained at all times.

Having a team leader can improve safety and situational awareness during a MET / Cardiac arrest and using this leader to perform the rhythm recognition and requesting the appropriate intervention decreases the chance of accidental shocks being delivered.

The use of the C.O.A.C.H.E.D acronym can assist with manual defibrillation and the charge on the chest technique (see below).

- **C**ompressions Continue
- **O**xygen away
- **A**ll else stand clear
- **C**harging the Defibrillator
- **H**ands off - are you safe?
- **E**valuate Rhythm
- **D**efibrillate or **D**isarm the defibrillator before recommencing CPR with the minimum of delay. (or assessing for a pulse if a potentially perfusable rhythm is present)

(If the first shock at 200J is not successful then it is reasonable to deliver all subsequent shocks at the highest energy level¹ available). Rescuers should not compromise on safety. Actions should be planned before stopping chest compressions. Rescuers should wear gloves during CPR attempts. If they are not immediately available this should not delay starting CPR. Wearing gloves may decrease the risk of receiving an inadvertent shock¹³.

3.11 Transportation of Patients after MET

At SCGH the MET CNS will liaise with the appropriate area and, bed management and ensure appropriate transport equipment is obtained. If the patient remains in MET calling criteria or at risk of further deterioration, this must include as a minimum,

- A doctor and nurse with advanced life support training.
- ECG monitoring and defibrillator
- Portable suction and oxygen and SaO₂ monitoring
- Bag valve mask plus waveform capnography if patient has been intubated
- Emergency medications deemed appropriate by the Medical MET Leader

At OPH if transfer to a tertiary centre is required, call 000 for a SJA paramedic team.

Related Documents

- [Australian Resuscitation Council Guidelines](#)
- [SCGOPHCG Recognising and Responding to Acute Deterioration Procedure](#)

References

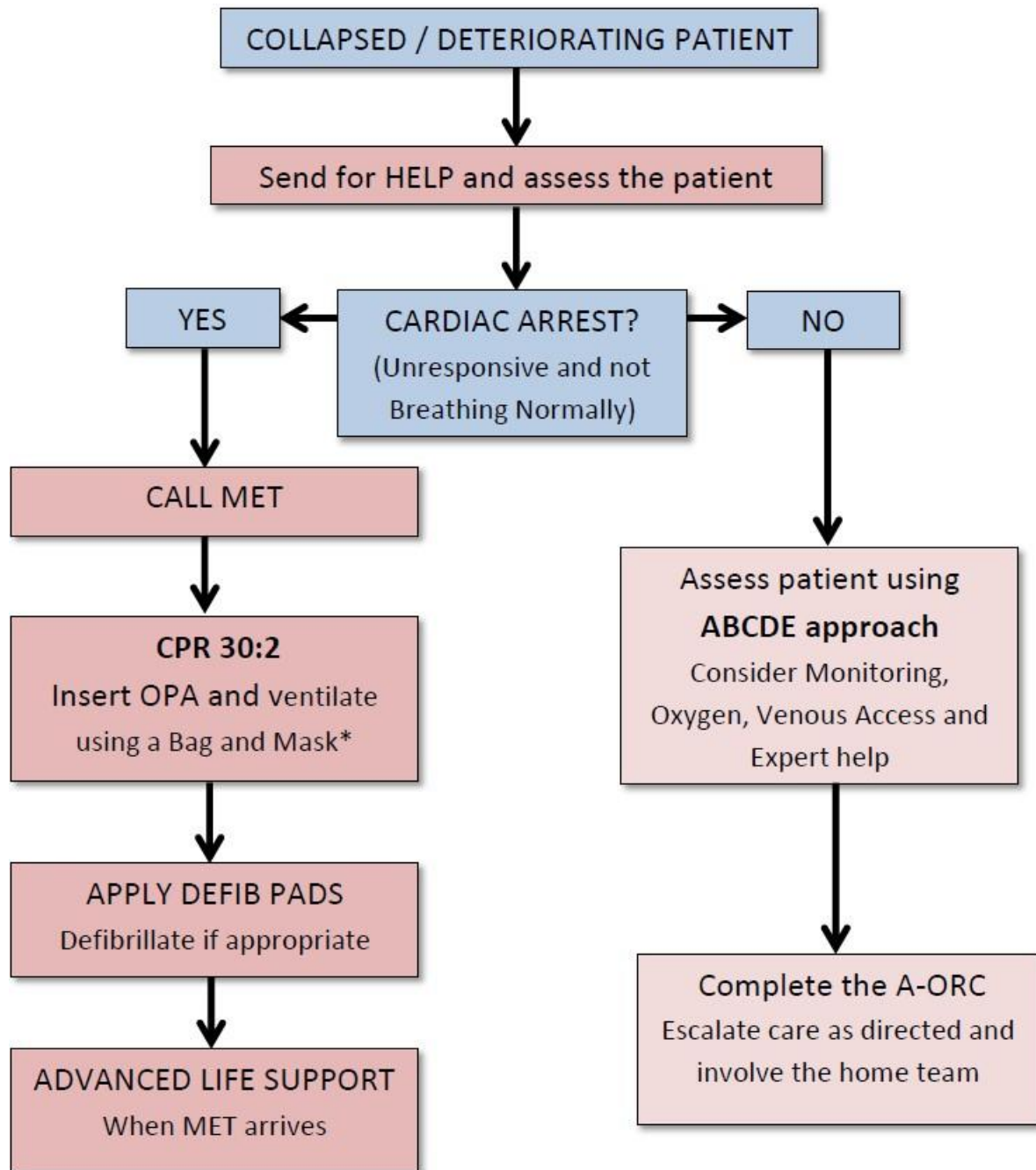
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Authorisation & Version Control

Policy Sponsor	SCGOPHCG Recognising and Responding to Acute Deterioration Committee Chair				
Policy Contact	Medical Emergency Team Educator, SCGH				
Last Reviewed:	22/01/2024	Last Amended:	19/12/2024	Review Due:	22/01/2027
Version:	2.1	Reference #	CPG010	Aboriginal Health ISD	Not required
Approved by:	SCOPHCG RRAD Committee SCGOPHCG Nursing Practice Guideline Committee			Date:	19/04/2024
Endorsed by:	SCGOPHCG Policy and Guideline Committee			Date:	17/05/2024
Applicable Standards	NSQHS Standards (Version 2): 				
Printed or personally saved electronic copies of this document are considered uncontrolled					
Document Version Control					
Date	Version	Author	Notes		
19/02/2024	2	MET Educator	Reviewed by NPG and RRAD Committees		
Feb 2026	2.1	CNS Patient Flow	Change made to align with current practice with the medication boxes on each trolley. Endorsed by CPGsC 19/02/2026.		

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

Appendix 1: Sequence for 'collapsed/deteriorating' patient in-hospital.



Basic Life Support

D

Dangers?

R

Responsive?

S

Send for help

A

Open Airway

B

Normal Breathing?

C

Start CPR

30 compressions : 2 breaths

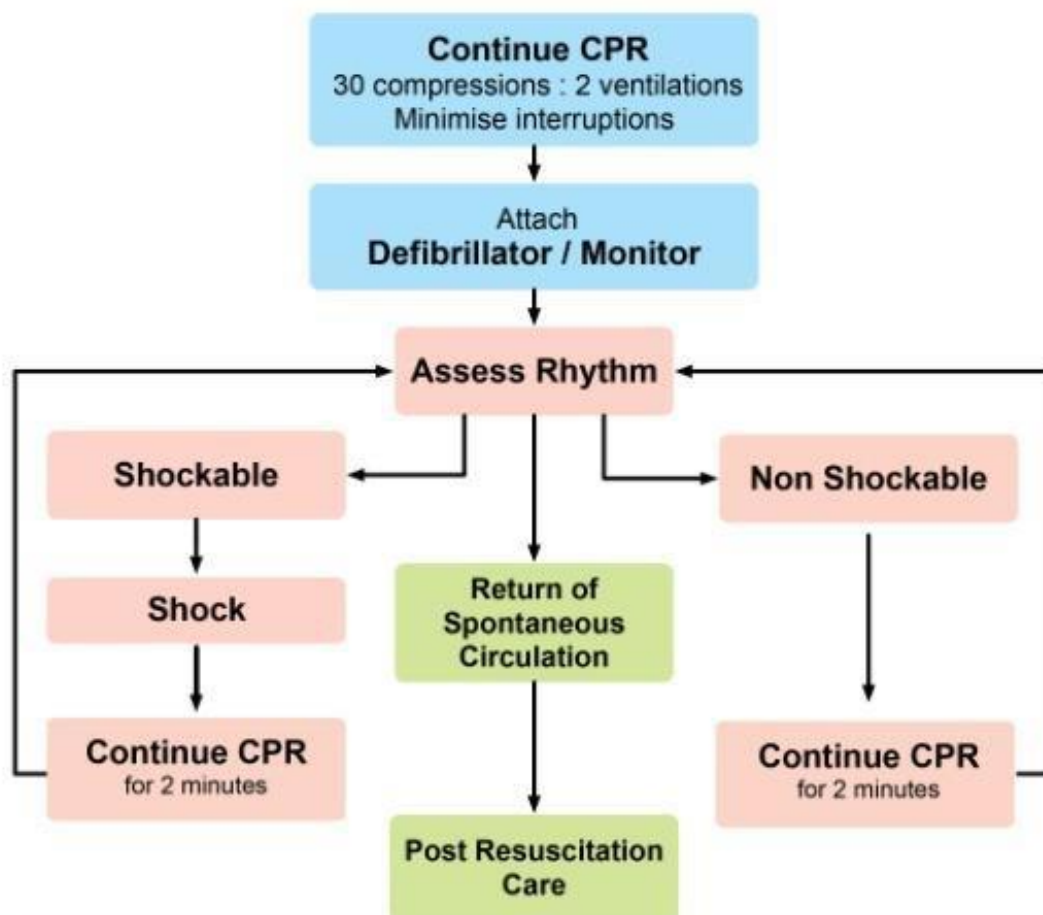
D

Attach Defibrillator (AED)

as soon as available, follow prompts

Continue CPR until responsiveness or normal breathing return

Adult Advanced Life Support



During CPR

Airway adjuncts (SGA or ETT)
Oxygen
Waveform capnography
IO or IV access
Plan actions before interrupting CPR
(e.g. charge defibrillator)

Drugs

Shockable

- Adrenaline 1 mg after 2nd shock (then every 2nd cycle)
- Amiodarone 300mg after 3 shocks

Non Shockable

- Adrenaline 1mg immediately (then every 2nd cycle)

Consider and correct

Hypoxia
Hypovolaemia
High or low electrolytes (K, Ca, Mg) & metabolic disorders
Hypothermia or hyperthermia
Tension pneumothorax
Tamponade
Toxins
Thrombosis (pulmonary or coronary)

Post Resuscitation Care

Re-evaluate ABCDE
12 lead ECG plus CXR
Treat precipitating cause
Aim for SpO₂ 94-98%, normocapnia, normoglycaemia
Temperature control



Reviewed August 2023



NEW ZEALAND
Resuscitation Council
WHAKAPUKEA. ATEARDE

Appendix 4: SCGH Observation and Response Chart

Date													Date
Time													Time
If respiratory rate ≥ 30 or ≤ 4 write value in box	Write ≥ 30 30-35 25-29 21-24 15-20 10-14 5-9 Write ≤ 4 ≥ 94 91-93 85-90 Write ≤ 94 ≥ 10 7-9 3-6 a 2												If respiratory rate ≥ 30 or ≤ 4 write value in box
Respiratory Rate (breaths / min)													Respiratory Rate (breaths / min)
If O_2 saturation ≥ 94 write value in box	Write ≥ 94 91-93 85-90 Write ≤ 94 ≥ 10 7-9 3-6 a 2												If O_2 saturation ≥ 94 write value in box
O_2 Saturation (%)													O_2 Saturation (%)
O_2 Flow Rate (L / min)													O_2 Flow Rate (L / min)
If heart rate ≥ 140 or ≤ 30 write value in box	Write ≥ 140 130a 120a 110a 100a 90a 80a 70a 60a 50a 40a Write ≤ 30 Write ≥ 200 190a 180a 170a 160a 150a 140a 130a 120a 110a 100a 90a 80a 70a 60a 50a 40a												If heart rate ≥ 140 or ≤ 30 write value in box
Heart Rate (beats / min)													Heart Rate (beats / min)
If systolic BP > 200 write value in box	Write ≥ 200 190a 180a 170a 160a 150a 140a 130a 120a 110a 100a 90a 80a 70a 60a 50a 40a												If systolic BP > 200 write value in box
Blood Pressure (mmHg)													Blood Pressure (mmHg)
(Exclude for systolic not diastolic)													(Exclude for systolic not diastolic)
If temperature ≥ 38.0 write value in box	Write ≥ 38.0 38.5-38.9 38.0-38.4 37.5-37.9 36.8-37.4 35.1-36.5 ≤ 35.0												If temperature ≥ 38.0 write value in box
Temperature (C)													Temperature (C)
Consciousness if necessary write patient before scoring	Alert To Voice To Pain Unresp.												Alert To Voice To Pain Unresp.
Blood Glucose Level Random (mmol / L) Consider Diabetic Chart	> 20 4-20 ≤ 4												> 20 4-20 ≤ 4
Passed Urine	Y/N												Passed Urine
Bowels Opened	Y/N												Bowels Opened
Pain Score None (0) - Worst (10)	Rest Movement												Pain Score None (0) - Worst (10)
Weight	Kg												Weight

USE PATIENT LABEL WHEN AVAILABLE

Room: _____

Phone: _____

Serial: _____

CHECK PATIENT ID AND AFFIX LABEL

Medical Emergency Team (MET) Call

Response Criteria

- Any observation is in a purple area
- Alway threat
- Respiratory or cardiac arrest
- Sudden fall in level of consciousness
- New drop in O_2 saturation $\geq 94\%$
- Repeated or Prolonged Seizure
- A medical review that has not been attended
- You (or a family member/carer) are worried about the patient but they do not fit the above criteria

Actions Required

- Place Emergency call
- Initiate ALS/BLS protocols
- If transfer is required, patient must be escorted by ALS staff member

Consider if your patient's deterioration could be due to sepsis, pneumonia, pulmonary embolus, a new cardiac arrhythmia, electrolyte imbalance, acute coronary syndrome, acute stroke or overdose/over sedation

Medical Review (Registrar/Consultant)

Response Criteria

- Any observation is in a red area
- New or unrelenting chest pain
- New or unrelenting shortness of breath
- Increased or unexpected fluid or blood loss
- Pain not relieved by current analgesia
- Drop in O_2 saturation $\geq 90\%$
- BGL > 4 or < 3.0
- Urine output $< 0.5\text{mL/kg/hr}$
- You (or a family member/carer) are worried about the patient but they do not fit the above criteria

Actions Required

- Record observations every 15 minutes
- Doctor to review patient within 30 minutes. If medical review not attended within 30 minutes, consider MET call
- GoSAR - Medical Review sticker
- If transfer is required, patient must be escorted

Senior Nurse Review

Actions Required

- Record observations at least once every 1 hour
- Senior Nurse must review patient
- Senior Nurse to consider medical review
- Senior Nurse Review sticker
- Manage fever, pain, fluids, blood loss or distress and O_2 requirement
- If transfer is required, patient must be escorted

Increased Surveillance

Actions Required

- Record observations at a minimum of 4 hours
- Inform Shift Coordinator
- Carry out appropriate interventions as prescribed
- Manage fever, pain, fluids, blood loss or distress and O_2 requirement

REASON FOR QUARTY OCCUPANCY DURING:

[illegible]

31R CHARLES GAIRDNER HOSPITAL

OBSERVATION AND RESPONSE CHART

USE PATIENT LABEL WHEN AVAILABLE

Signature _____

Signature _____

Specialist _____

Other Charts in Use

☐ Alcohol/Drug Withdrawal Chart
☐ Fluid Balance Chart
☐ Neurovascular Chart

☐ Regional/Epidural/Patient Controlled Analgesia Chart
☐ Other _____

Refer to above charts for additional instructions.

Clinical Management Plan for Altered Parameters

If adjusted parameter ranges are to be acceptable for the patient's clinical condition, the table below including range, escalation required along with rationale and justification must be completed.

Observation	Parameter Range	Escalation Required (Please indicate SP-R / MR / SST)
Respiratory Rate	-	
Oxygen Saturation	-	
Systolic BP	-	
Heart Rate	-	
Temperature	-	
Other (specify)	-	

Clinical Rationale & Justification (Registrar/Consultant)

Note: Modification WILL NOT be valid unless clinical rationale and justification are documented

Date / Time	Doctor's name	Doctor's signature

Parameters must be reviewed every 72 hours. If the clinical rationale and management plan remains unchanged the parameters can be reinsordsed.

Date / Time	Doctor's name	Doctor's signature
Date / Time	Doctor's name	Doctor's signature
Date / Time	Doctor's name	Doctor's signature
Date / Time	Doctor's name	Doctor's signature

☐ Time-Limited Post-Procedural Modifications in place (refer to Page 8)

SIR CHARLES GAIRDNER HOSPITAL		USE PICTURE LABEL WHEN AVAILABLE					
OBSERVATION AND RESPONSE CHART		Surname					
		Forename					
		Gender	DOB				
CHECK PATIENT ID AND AFFIX LABEL							
Additional Observations							
Date							
Time							
Urinalysis	Specific gravity						
	pH						
	Leucocytes						
	Blood						
	Nitrate						
	Ketones						
	Bilirubin						
	Urobilinogen						
	Protein						
Glucose							
Time Limited Post-Procedural Modifications - Valid for a maximum of 4 hours							
If adjusted parameter ranges are to be acceptable for the patient's clinical condition after a procedure or anaesthetic, the table below including range, escalation required along with rationale and justification must be completed.							
Observation	Parameter Range	Timeframe if < 4hrs					
Respiratory Rate	-						
Oxygen Saturation	-						
Systolic BP	-						
Heart Rate	-						
Temperature	-						
Other (specify) _____	-						
Instructions when observations fall outside the above range (Registrar/Consultant)							
Contact Team for Medical Escalation:		Ext #					
Date / Time	Doctor's name	Doctor's signature					

Appendix 5: ABCDE systematic assessment chart

Airway

Assess

- Is the patient talking?
- Are there abnormal sounds e.g. stridor, gurgling

Consider

- Suctioning
- Oxygen therapy
- Airway manoeuvres & or adjuncts as indicated

Breathing

Assess

- Respiratory rate
- O2 saturations
- Chest movement and effort of breathing

Consider

- Auscultation and ordering a Chest X-ray
- Oxygen therapy
- Supplemental breaths using bag and mask

Circulation

Assess

- Pulse rate/rhythm, blood pressure
- Patient colour/capillary refill time
- Blood/fluid loss
- IV access, fluids and ECG monitoring

Consider

- 12 lead ECG

Disability

Assess

- Level of consciousness using AVPU scale
- Mental Health status (Appearance, Behaviour, Conversation/Cognition)
- Drug chart and relevant documentation

Consider

- Monitoring blood glucose
- GCS & Pupillary response

Exposure

Assess

- Temperature
- Head to toe examination

Consider

- Checking peripheral pulses/wounds/drains etc.
- Urine output
- Pain score

All findings are to be recorded on the relevant documents such as the ORC. Escalation of care must be activated according to local policy or if there are concerns about the patient.

Please note: If MET criteria is identified while conducting a patient assessment - activate MET call

Appendix 6: Restocking Resus Trolley Process

